



Mental health - visual observations and specialling

1. Background

1.1. About this document

South Metropolitan Health Service (SMHS) Mental Health is committed to providing safe clinical care that optimises the clinical and personal recovery of the patient, is trauma informed and least restrictive. In the mental health inpatient setting visual observations of a patient are a key tool used to promote a safe environment for all people. Visual observations are undertaken for all mental health patients as clinically indicated to maintain a patient's safety, the safety of others and as a therapeutic intervention for identified clinical need. This procedure guides staff in identifying the least restrictive level of visual observations required for a patient and the provision of safe and supportive visual observations.

This document is an amalgamation of, and replaces:

- FSFH Patient Observation – Specialling and Close observations
- RkPG Observations: Safe and supportive

1.2. Key definitions

Term	Definition
Personal Support Person (PSP)	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult Mental Health Act 2014 s.7 . Some consumers may use the term kin to refer to a family member.
Risk assessment	The gathering and analysis of clinical information related to risks to the consumer and/or others. It includes background (static) risk factors interacting with acute (dynamic) risk factors and the consumers current diagnosis and Mental State Examination to provide a risk assessment framework for clinical judgment and decision making.

Special	Refers to the allocation of a designated nursing staff member to provide Level 1 or 2 patient observations, and assessment and monitoring of mental health related parameters. The type of nurse required to 'special' the patient can vary and includes Registered Nurses, Enrolled Nurses and Assistants in Nursing (AIN).
Trauma informed care	Is a strengths-based service delivery approach 'that is grounded in an understanding of and responsiveness to the impact of trauma, that emphasises physical, psychological and emotional safety for both providers and survivors, and that creates opportunities for survivors to rebuild a sense of control and empowerment'. ¹ The core trauma informed principles are safety, trustworthiness, choice, collaboration, empowerment and respect for diversity.
Visual observations	An active process of observing a patient through interaction and therapeutic engagement to benefit treatment, support recovery and enhance safety. Observation grounded in therapeutic engagement and person-centred care enables the purposeful gathering of clinical information to reach a comprehensive understanding of the patient's needs.

2. Safe and therapeutic practice requirements

2.1. Person centred care

- SMHS mental health recognises and fully supports the need for the patient and/or their PSP to:
 - know and exercise their healthcare rights
 - be engaged in their health care
 - have access to information about treatment options and agreed treatment plans
 - participate in treatment decisions
- Consideration will be given to the severity of the patient's illness, their level of engagement and understanding, vulnerabilities, potential for impulsiveness, presence of suicidal ideation and state of emotional arousal when determining the level of observations.
- Patients must be informed of the of their level of observation and why that level of observation is required, explaining that observations continue overnight and discussing strategies for managing observations in the least restrictive and intrusive manner.
- For those patients who have sensory sensitivities which may be heightened during a mental health crisis consideration will be given to minimising sensory overload.
- Patients will be informed when the level of observation is increased or decreased and given explanation of the reason why changes have been made.
- The patient's visual observation requirements must be documented in the Treatment Support and Discharge Plan (TSDP) including the level of observation, offering choices where possible and recognising potential triggers. The TSDP will be regularly reviewed with the patient and updated.

- All Aboriginal patients must be offered cultural support through Aboriginal Mental Health Liaison or Aboriginal Health Liaison Officers to ensure culturally appropriate communication with family and kin and to assist in navigating culturally sensitive situations.

2.2. Safe practice

- Visual observations of a patient must be documented on the relevant site-based medical record form:
 - FSFH Patient Observation Chart – Mental Health
 - RkPG Safe and Supportive Observation Chart Category One & Two
 - RkPG Mental health 1:1 Specialising form
 - RkPG Safe and Supportive Observation Chart Category Three
- A patient does not need to be under the [Mental Health Act \(MHA\) 2014](#) to require a special and patients under the [MHA 2014](#) do not automatically require a special.
- With the consent of the patient, the next of kin, family or significant other will be informed of any marked deterioration in a patient's mental state and the rationale for specialising the patient will be documented in the patient's medical record.
- Visual observations of a patient must be completed as ordered see Section 3.2. Where there is an unplanned delay in completing visual observations, the reason will be documented on the patient observation chart
- Observation times must not be written in advance (pre-populated)
- The actual time the visual observation is completed must be the time documented
- Observations must not be documented retrospectively
- Patients' must have their respiration rate documented on the patient observation chart at a minimum of one hourly intervals when they are asleep, lying down, resting with eyes closed or when clinically indicated.
- Nursing staff may implement a higher level of observation based on a change in the patient's clinical presentation. The treating team will be informed, and rationale documented in the patient's medical record.
- The level of observations in place before or during the admission may not be reduced by nursing staff until reviewed and approved by a Medical Officer and documented in the patient's medical record.
- Nursing staff completing visual observations will endeavour to support and engage patients to build therapeutic alliances while being responsive to patient needs and changes in their physiological and mental state.
- Verbal clinical handover using iSoBAR will occur between each nursing shift and include the patient's current level of observations. Completed visual observations must be confirmed when handing over to the next allocated nursing staff member and co-signed on the patient observation chart.
- Whenever practicable (e.g. patient is awake) the oncoming nurse will introduce themselves to the patient under observation.
- Following handover, it is the responsibility of the allocated nurse taking over care to confirm the level of visual observation from the medical record and collated information from the treating teams.

3. Procedure

3.1. Clinical risk assessment

- All patients will have a risk assessment and management plan (RAMP) completed by member of the multidisciplinary team (MDT) immediately prior to or on admission to a mental health inpatient ward.
- The RAMP will be documented in webPSOLIS and a copy made available in the patient's medical record.
- Whilst on the inpatient ward the patient's risk will be reviewed and documented in the medical record every shift and whenever there is a change in clinical presentation
- Patients may require specialising if they are assessed as at increased risk of:
 - harm to self
 - harm to others
 - absconding
 - harm from others
 - accidental harm e.g. falls
 - any other identified concerns e.g. physiological deterioration.
- For Aboriginal patients' cultural disconnection, grief and loss, racism and historical trauma may contribute to increased risk. These factors must be considered in the risk assessment and culturally appropriate supports offered.
- Clinicians will enter an alert into PSOLIS when significant risk of aggression, self-harm or absconding is identified.

3.2. Levels of observation

- The treating or duty psychiatrist (or medical delegate) will review the patient (face to face) and in consultation with the Clinical Nurse Specialist (CNS), Nurse Unit Manager (NUM) or delegate (Shift Coordinator) determine the level of visual observation required, level of access to family and any environmental risk mitigation strategies required and document in the patient's medical record.
- Decisions about the level of visual observation required will be based on:
 - the assessment of the patient's clinical needs and level of risk
 - the safety of the patient
 - the safety of other patients and staff
 - staffing levels required for the safe management of all patients

Table 1: Visual observation levels

Level of Observations	Observation requirements
Level 1: Within arm's length	• The special will remain within an arm's length of the patient at all times. • Patient not to leave the ward area • Special remains with patient during interviews or clinical interventions

	Patients allowed brief moments of privacy if safety can be maintained. Refer to Section 3.4 Patient dignity and privacy.
Level 2: Within eyesight	- Special to maintain constant line of sight of the patient - Patient not to leave the ward area - Special remains with patient during interviews or clinical interventions - For prompt response when needed, nursing staff (special) to be located in ward area, not within nursing station (ie. visual line of sight must not be from within the nursing station) - Patients allowed brief periods of privacy if safety can be maintained. Refer to Section 3.4 Patient dignity and privacy.
Level 3: Intermittent	The mental state and location of the patient is checked and documented in the stipulated time intervals e.g. every 15 or 30 minutes
Level 4: General visual	The mental state and location of the patient is checked and documented every 60 minutes.

- The least restrictive form of observation will always be applied while still managing the assessed level of risk.
- On admission to an inpatient ward, unless otherwise documented, a patient will be commenced on level 3 intermittent visual observations every 15 minutes until reviewed by a medical officer and the level of observation confirmed in the medical record.
- All patients admitted to a mental health inpatient ward will have a minimum of level 4 general visual observations every 60 minutes for the duration of the admission.
- On initial cessation of specialising a patient, a minimum of level 3 Intermittent visual observations will apply until further medical review.
- Where possible staff will provide an explanation to the patient and carer or PSP regarding the level of visual observation required and involve the patient in care planning including treatment aims/goals and any safety issues.
- Patients on level 3 & 4 visual observations may be granted leave at the discretion of the treating consultant.

Refer to:

[SMHS Granting and management of leave for mental health inpatients](#)

[FSFH Patient leave and escort – Mental Health](#)

- Where a patient is not able to be located at the time of observation staff will immediately report to the Shift Coordinator.

Refer to:

[SMHS Mental health consumers who are missing or absent without leave](#)

FSFH Patient leave and escort – Mental Health

3.3. Requesting level 1 & 2 observations - specialling

- A recommendation or request for a special can be made by any member of the MDT
- The rationale for requesting a special, the required skills and experience of the special and the observation level required will be documented in the patient's medical record.
- Once a decision is made to institute or alter a level 1 or 2 special the treating or duty psychiatrist (or medical delegate) must document whether the special will be 'at arm's length' or 'within eyesight'.
- Consideration must be given to 'line of sight' specialling where 'arm's length' specialling is likely to negatively impact the patient's mental state or put the nursing staff member at risk
- Patients under the age of 18 years admitted to an adult inpatient unit must have strategies to keep the child safe documented on a Segregation of Children from Adult Inpatients Notification Form [MHA 2014](#) s. 303. All patients under 18 years must be specialised at a minimum of Level 2 visual observations if admitted to a SMHS mental health adult inpatient ward.
- In line with trauma informed care when requesting a special consideration will be given to the patient's:
 - gender
 - ethnicity
 - cultural and religious needs
 - age and developmental stage
- For Aboriginal patients the allocation of staff for specialling should consider cultural safety.
- Specials will be requested, approved and reviewed in accordance with site-based policies.

Refer to:

[FSFH Specialling and close observations: Provision of a Special for Patients in Non-Psychiatric clinical areas guideline](#)

[RKPG Resource Management for patients requiring increased observations policy and procedure](#)

[PHC Specialling and observations: Provision of a special for patients in non-psychiatric clinical areas guideline](#)

3.4. Level 1 & 2 observations – specialling a patient

- The Shift Coordinator will:
 - determine the most appropriate nursing staff to provide specialling based on the staff members skills knowledge and experience appropriate to the needs of the patient
 - orientate the nursing staff member to the ward
 - inform the nursing staff member of timeframe for specialling and how to escalate any concerns
 - promote an active role in engaging positively with the patient
 - ensure clinical handover includes the rationale for specialling, whether it is level 1 or level 2 and any specific patient needs
- Care will be individualised to the patient being specialised. The Consultant Psychiatrist in collaboration with the MDT will consider the therapeutic value and impact of interventions

alongside potential risks and document specific instructions in the medical record regarding the patients:

- access to therapeutic groups
- attendance at ward programs
- which patients can be seen by Allied Health without their nurse special
- access to leave
- Where the Consultant Psychiatrist and MDT consider the patient would benefit from individual therapy with a member of the Allied Health team the nurse special must always be able to observe the patient.
- When a patient being specialised receives visitors:
 - the safety needs of any visiting children must be given priority
 - children must be in the immediate supervision of a responsible adult (not the patient or the special) during any visit
 - visitors must not at any time be left alone with a patient who is being specialised unless otherwise documented by the treating consultant.
 - during visits the duty of care to provide an adequate level of patient observation remains with the nurse who is specialising the patient.
- The allocated nurse caring for patient on a special level 1 or 2 for all or part of their shift will make an entry in the patient's medical record. This documentation will detail the nurse's observation of the patient's mental state, behaviour, content of conversation, any therapeutic interventions and all other relevant patient information.
- The requirement for a special will be reviewed every shift. Following patient reassessment the NUM/CNS/After Hours CNM may recommend discontinuing a special to the medical team who has the delegated authority to discontinue the special.
- The decision to continue or cease the 'special' will be documented in the patient's medical record and the time noted on the observation chart.

3.5. Patient dignity and privacy

- When a patient is toileting or showering their needs for safety and privacy may conflict. Concessions to the patient's need for privacy will always be restricted by the need to maintain safety.
- Interventions such as "hearing" rather than "seeing" may be appropriate at such times, thereby allowing brief moments when the patient has a degree of privacy.
- If a patient is on **level 1 observations** and requires the use of the toilet/shower inform the patient, the following will occur and why:
 - Every 30 seconds the 'special' is to knock on the door or call out to the patient requesting a response to confirm the patient's wellbeing.
 - Failure of the patient to respond will result in the door being opened to confirm the patient's welfare.
 - If the patient has deteriorated physically or psychologically, follow site-based guidelines for recognising and responding to acute deterioration.
- If a patient is on **level 2 observations** and requires the use of the toilet and/or shower inform the patient, the following will occur and why:

- Every three minutes the nurse special is to knock on the door or call out to the patient requesting a response to confirm the patient's wellbeing. The patient has 30 seconds to respond.
- Failure of the patient to respond will result in the door being opened to confirm the patient's welfare.
- If the patient has deteriorated physically or psychologically, follow site-based guidelines for recognising and responding to acute deterioration.

4. Security specials

- A nurse special must always be in place when there is a security special, unless at RkPG where there will be two nurses rostered for four patients.
- The NUM or After-Hours Clinical Nurse Manager (CNM) will organise a security special through the relevant site-based security service informing them of the requirements prior to 10:00hrs.
- The treating team, NUM and Shift Coordinator will review the requirement for security specials every 24 hours prior to 10:00hrs where possible
- Ward staff will:
 - orientate the security special to the ward
 - inform the security special of the role of the nurse as the clinical lead
 - provide clear directions regarding the role of security special
 - advise of the level of observation required
 - advise how to escalate care
 - provide a duress pendant to the security special
 - in open treatment areas ensure the patient and security special are in the line of sight of clinical staff
 - inform of Code Black procedure as per relevant site.
- Security Specials will:
 - ensure the nurse is not left alone with the patient
 - have a current Aggression Prevention Intervention competency or equivalent
 - inform the Shift Coordinator if a toilet break is required
 - ensure security special checks have been completed and as required
 - continue to provide special until advised no longer required by the relevant security manager.
- The NUM, CNS, or After Hours CNM will notify the security service when to cease the special, they will then liaise with the provider of the special regarding cancellation.

5. Compliance Monitoring

Each SMHS MH site or Service Director/Co-Director is to ensure compliance with this policy. Compliance monitoring methods may include the following:

- Number and type of specials used
- Monitoring of clinical incidents related to incorrect documentation on paper medical record and/or failure to recognise significant change in patient status
- Point prevalence survey of visual observations will be completed according to the SMHS MH Audit Schedule

- All inpatients to be encouraged to complete a Your Experience of Service patient survey where they can provide feedback on their experience of being observed or raise and/or concerns.

6. Legislation

There are no legislative obligations relating to this procedure for which a statutory penalty is imposed for a breach.

- [Mental Health Act 2014](#)

7. Related Documents

7.1. Department of Health policies

- [MP 0015/21 State-wide Standardised Clinical Documentation for Mental Health Services](#)
- [MP 0012/16 Mental Health Consumers Who are Missing or Absent Without Leave Policy](#)
- [Principles and best practice for the care of people who may be suicidal](#)
- [Principles for the care of people who may be at risk of violent or aggressive behaviour](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- [MP 0122/19 Clinical Incident Management Policy](#)
- [MP 0095/18 Clinical Handover Policy](#)

7.2. SMHS policies

- [Granting and management of leave for mental health inpatients](#)
- [Mental health consumers who are missing or absent without leave](#)
- [Mental Health Clinical Incident Procedure](#)

7.3. Other documents

- [Chief Psychiatrists Standards for Clinical Care](#)
- [WA Aboriginal Health and Wellbeing Framework 2015-2030](#)
- [National Aboriginal and Torres Strait Islander Suicide Prevention Strategy 2025-2035](#)

7.4. References

1. Hopper, E. K., Bassuk, E. L., and Olivet, J. (2010). Shelter from the storm: Trauma-informed care in homeless service settings. The Open Health Services and Policy Journal, 3, 80-100

7.5. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP.V1	28 October 2025	24 October 2025	24 October 2028	October 2028	Original version
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7.6. Approval

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This document can be made available in alternative formats on request.

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